

So You (Think) You Have BPD?

Therapy options, the skills that underpin them, and where to turn when things get hard. Written for the person living it.



SECTION ONE

Exploring therapy options

If you find yourself navigating intense, fast-shifting emotions, worrying about being abandoned, or feeling unsure about your sense of self, you might consider looking into borderline personality disorder. While these experiences can feel overwhelming at times, you might find comfort in knowing that BPD responds very well to practical, structured therapies designed to help you regain balance and feel more grounded.

You might want to start by exploring **Dialectical Behavior Therapy (DBT)**, which was created specifically to help manage BPD symptoms. The core idea of “dialectics” is about finding a balance between two concepts: accepting where you are right now, while gently trying out new and healthier ways to cope.

The four DBT skill areas

- **Core mindfulness.** You might practice observing, describing, and engaging in the current moment without judging yourself. Bringing awareness to the present can help anchor you when feelings start running high.
- **Distress tolerance.** You could try these practical tools to get through intense emotional moments without making things harder on yourself or turning to self-harm. They offer gentle ways to soothe yourself, take a break from stress, and accept difficult situations step by step.
- **Emotion regulation.** Consider using these techniques to help you name what you are feeling, spot your emotional triggers, and make strong emotional reactions feel a bit more manageable.
- **Interpersonal effectiveness.** You might use these strategies when you want to express your needs, set clear boundaries, or say no while protecting your self-respect and your relationships.

SECTION TWO

Experimenting with distress tolerance

Trying out distress tolerance skills in your daily routine can help break loops of overthinking, release tension in your body, and bring back mental clarity when stress peaks. Here are a few ways you might consider approaching them.

Matching skills to how you feel

- **Low to moderate stress.** Subtle options like mindfulness practice, calm breathing exercises, or light emotional check-ins can work nicely.
- **High distress and crisis.** If you feel extremely overwhelmed, you might consider engaging strong physical senses, such as holding an ice cube, tasting something sour, or using temperature shifts,

to help reset your nervous system and bring your attention back to the present.

Giving yourself time to adjust

- **Initial awkwardness is normal.** You might experience some pushback or feel awkward when trying these tools for the first time, which is completely natural. It often takes a bit of gentle trial and error to see which skills feel right for you.
- **Practice in calm moments.** You might find it helpful to practice when you are feeling settled, making it much easier to lean on these tools automatically when things get tough.

SECTION THREE

Other pathways to consider

Beyond DBT, you might also want to look into other well-supported therapy approaches to see if one aligns better with your personal needs.

- **Schema Therapy.** You might explore this approach if you want to look at deeper patterns from earlier in life, especially if you are working through past trauma or PTSD.
- **Mentalization-Based Treatment.** You could consider this if you would like to get better at reading your own feelings and understanding other people's intentions, helping create space between an emotion and your reaction.
- **Transference-Focused Psychotherapy.** You might look into this option to work closely with a therapist on relationship patterns in the moment, helping build a stronger sense of identity.
- **Brief DBT programs.** If long-term programs feel out of reach, you might consider shorter skills groups, under sixteen weeks, to learn practical coping tools.



Hard days do not undo the work. Recovery in the research is measured over months and years, not over a single afternoon.

SECTION FOUR

Resources and crisis support

If you need help right now

- **988 Suicide & Crisis Lifeline.** Call or text **988**. Free and confidential, 24/7 in the US and Canada.
- **Crisis Text Line.** Text **HOME** to **741741** to reach a crisis counselor, 24/7.
- **The Trevor Project** (LGBTQ youth). Call **866-488-7386** or text **START** to **678-678**, 24/7.

Learning more and finding people

- **National Education Alliance for Borderline Personality Disorder.** Educational materials, webinars, and information on evidence-based BPD resources.
- **Emotions Matter.** A non-profit advocating for BPD awareness and offering peer support groups.
- **International Society for the Study of Personality Disorders.** Clinical and educational information about personality disorders.

This guide is not a diagnosis.

Only a qualified clinician can diagnose BPD, and recognizing yourself in a list of criteria is not the same as meeting them. What this guide can do is give you the vocabulary to describe what you are experiencing to someone who can help.



ABOUT THE GUIDE

Meet Marley

This project started with one family trying to make sense of a diagnosis.

Marley grew up inside a family where emotions ran high and nobody had a word for why. That question led her to spend a summer buried in clinical research on borderline personality disorder, and to build this series of guides for other families doing the same thing hers did.

Each guide is written from the peer-reviewed literature rather than from anecdote, and every claim is traceable to the sources listed at the back. They are free, and they are meant to be shared with the people who need them.

“Nobody in my family had the language for what was happening. These guides are the thing I wish someone had handed us.”

SOURCES

Works cited

1. Chapon, F., et al. (2018). Dialectical behavior therapy as treatment for borderline personality disorder. [pmc.ncbi.nlm.nih.gov/articles/PMC6007584/](https://pubmed.ncbi.nlm.nih.gov/articles/PMC6007584/)
2. Crisis Text Line. (2024). *Free 24/7 crisis support via text*. crisistextline.org
3. Australian BPD Foundation. (2024). *Diagnostic criteria for Borderline Personality Disorder*. bpdfoundation.org.au/diagnostic-criteria.php
4. Emotions Matter. (2024). *BPD advocacy and peer support resources*. emotionsmatterbpd.org
5. Flesch, E., et al. (2021). How do patients with borderline personality disorder experience distress tolerance skills in the context of dialectical behavioral therapy? A qualitative study. *BMC Psychiatry*. [pmc.ncbi.nlm.nih.gov/articles/PMC8205175/](https://pubmed.ncbi.nlm.nih.gov/articles/PMC8205175/)
6. International Society for the Study of Personality Disorders. (2024). isspd.net
7. Linehan, M. M. (1993). *Cognitive-Behavioral Treatment of Borderline Personality Disorder*. Guilford Press.
8. Löffler, A., et al. (2018). Evidence-based psychodynamic therapies for the treatment of patients with borderline personality disorder. *Clinical Psychology in Europe*. doi.org/10.32872/cpe.v1i2.30639
9. Meuldijk, D., et al. (2018). PRO*BPD: effectiveness of outpatient treatment programs for borderline personality disorder. *BMC Psychiatry*. doi.org/10.1186/s12888-018-1905-6
10. National Education Alliance for Borderline Personality Disorder. (2024). borderlinepersonalitydisorder.org
11. Substance Abuse and Mental Health Services Administration. (2024). *988 Suicide & Crisis Lifeline*. samhsa.gov/find-help/988
12. The Trevor Project. (2024). thetrevorproject.org
13. Vasiljevic, M., et al. (2025). Feasibility and efficacy of brief DBT intervention for adults with borderline personality disorder or traits: a systematic review. [pmc.ncbi.nlm.nih.gov/articles/PMC12409768/](https://pubmed.ncbi.nlm.nih.gov/articles/PMC12409768/)
14. Zainal, N. H., et al. (2021). Implementation of brief dialectical behavior therapy skills training among borderline personality disorder patients in Malaysia. *BMC Psychiatry*. [pmc.ncbi.nlm.nih.gov/articles/PMC8489091/](https://pubmed.ncbi.nlm.nih.gov/articles/PMC8489091/)

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